



Stroke

Assessment

Pediatric Pearls:

- Use pediatric dosing of medications or electrical therapy for a pediatric patient < 37 kg and as defined by HandTevy, hypotension is defined as SBP < 70 + (age in years x 2) mmHg

Signs & Symptoms:

- Altered mental status
- Weak / Paralysis
- Blindness or other sensory loss
- Aphasia / Dysarthria
- Syncope
- Vertigo / Dizziness
- Vomiting
- Headache
- Seizures
- Respiratory pattern change
- Hyper/hypotension

Differential:

- Altered mental status
- Transient Ischemic Attack (TIA)
- Seizure
- Hypoglycemia
- Hypoxia / Hypercarbia
- Stroke
- Thrombotic / Embolic (85%)
- Hemorrhagic (15%)
- Tumor
- Trauma
- Atypical migraine

Clinical Management Options

P	P	P	P	P	P	• Oxygen titrated and PRN • BGL assessment • Cincinnati Pre-hospital Stroke Screen (CPSS) Assessment • If CPSS+, then specifically assess for motor weakness. ◦ If weak, then Visual Disturbance, Aphasia, and Neglect (VAN) Score • Basic airway management
L	L	L	L	L	L	• Positive CPSS & Glucose > 50 and last known well ≤ 23 hours, then declare Stroke Alert and < 15 minute-on-scene time • Positive CPSS, weakness, +VAN & Glucose > 50 and last known well ≤ 23 hours, then declare LVO Alert and < 15 minute-on-scene time • Place 4 & 12 lead ECGs, EtCO₂ monitoring
1	2	3	4	5	6	• Vascular access, 2nd vascular access if time and patient conditions permit • Monitoring and interpretation of ECG

Consult Online Medical Control As Needed

Pearls:

- Refer to drug formulary charts for all medication dosing for both adults and pediatric patients.
- A patient may have a positive CPSS without weakness, which is still a Stroke Alert. A LVO Alert requires weakness and positive VAN.
- Stroke patients are transported per Stroke Alert or LVO Alert criterion and Hospital Transport Grid.
- #FILM – Unit #, Family Contact Info, Recent Interventions, Last Known Well, Medications (antiplatelet & anticoagulants)
- Last known well is defined as the last time the patient was seen symptom free; example: Awakening with stroke symptoms would be defined as an onset time of the previous night when the patient went to bed symptom free.
- Whenever possible, a family member should accompany the patient to the hospital to provide a detailed history or provide the hospital with the name and contact information of someone who can.
- The differential list on the Altered Mental Status guideline should be considered.
- Be alert for airway problems (swallowing difficulty, vomiting).
- Hypoglycemia can present as a localized neurological deficit, especially in the elderly.
- Blood samples for performing glucose analysis should be obtained through a finger-stick (heel for infants). Venous blood samples may produce artificially high glucose values and should be avoided.
- IV access is preferred sizes 20g or 18g with AC placement.